Uterine/endometrial receptivity testing

There is insufficient evidence supporting the safety and efficacy of uterine receptivity testing and/or treatment. More studies are needed to support improved outcomes such as successful pregnancies with delivery of liveborn children.

In a recent study by Hoogenhuijze et. al. (2021) the SCRaTCH trial was a non-blinded randomized controlled trial in women with one unsuccessful IVF/ICSI cycle and assessed whether a single endometrial scratch using an endometrial biopsy catheter would lead to a higher live birth rate after the subsequent IVF/ICSI treatment compared to no scratch. The study took place in 8 academic and 24 general hospitals. Participants were randomized between January 2016 and July 2018 by a web-based randomization program. Secondary outcomes included cumulative 12-month ongoing pregnancy leading to live birth rate.

After the fresh transfer, 4.6% more live births were observed in the scratch compared to control group (110/465 versus 88/461, respectively, risk ratio (RR) 1.24 [95% CI 0.96-1.59]). These data are consistent with a true difference of between -0.7% and +9.9% (95% CI), indicating that while the largest proportion of the 95% CI is positive, scratching could have no or even a small negative effect. Biochemical pregnancy loss and miscarriage rate did not differ between the two groups: in the scratch group 27/153 biochemical pregnancy losses and 14/126 miscarriages occurred, while this was 19/130 and 17/111 for the control group (RR 1.21 (95% CI 0.71-2.07) and RR 0.73 (95% CI 0.38-1.40), respectively). After 12 months of follow-up, 5.1% more live births were observed in the scratch group (202/467 versus 178/466), of which the true difference most likely lies between -1.2% and +11.4% (95% CI).

This study was not blinded. Knowledge of allocation may have been an incentive for participants allocated to the scratch group to continue treatment in situations where they may otherwise have cancelled or stopped. In addition, this study was powered to detect a difference in live birth rate of 9%.

Regulatory

Infertility treatment is a procedure and, therefore, not subject to FDA regulation. However, any medical devices, drugs, biologics, or tests used as a part of this procedure may be subject to FDA regulation.

Applicable Codes

The following list of codes are intended for reference purposes only, is not an all-inclusive code listing. Applicable codes include but are not limited to:

Service Type	Code	Description
Diagnosis		
Antisperm Antibodies	89325	Antisperm antibody test